



Guidelines: Commercial. Treatment: C2 to C3 and C4 to C5 laminectomy. Diagnosis: Neck pain.

Prior Authorization Summary for C2–C3 and C4–C5 Cervical Laminectomy

Request Overview

Treatment:

CPT:

ICD-10:

Medical History:

Lacking documentation of neurologic deficits (radiculopathy or myelopathy)

Lacking failure of conservative therapy

Lacking imaging evidence correlating to symptoms (e.g., MRI/CT showing compression at symptomatic levels)

Prior Authorization Required:

Medical Necessity Criteria:

According to commercial guidelines, cervical laminectomy is typically considered medically necessary when ALL applicable criteria are met:

- ☐ Clinical indications
 - ☐ Objective neurologic involvement such as:
 - ☐ Cervical radiculopathy with corresponding dermatomal pain, sensory changes, reflex change, or motor weakness; or
 - ☐ Cervical myelopathy (gait disturbance, hand clumsiness, hyperreflexia, pathologic reflexes, bowel/bladder changes), or
 - ☐ Progressive or severe motor deficit attributable to cervical root/cord compression
 - ☐ AND imaging (MRI or CT/myelogram) demonstrating cord and/or nerve root compression at the planned surgical level(s) that correlates with exam findings and symptoms
- ☐ Conservative therapy (for non-urgent cases)
 - ☐ Failure of a structured course of conservative management for at least 6 weeks within the last 6 months, typically including:
 - ☐ Activity modification AND
 - ☐ Analgesics/anti-inflammatories and/or neuropathic pain medications AND
 - ☐ Supervised physical therapy and/or a documented home exercise program

- ☐ Note: Conservative therapy may be bypassed for red flag/urgent indications (e.g., acute or progressive myelopathy, rapidly progressive motor deficit, or acute cord compression)
- ☐ Level selection and scope
 - ☐ Symptoms and exam findings must localize to the level(s) to be decompressed
 - ☐ Imaging must confirm pathology at each requested level (e.g., C2–C3 and C4–C5)
 - ☐ For noncontiguous levels, provide specific rationale for each treated level
- ☐ Not primarily for axial neck pain
 - ☐ Laminectomy is generally not considered medically necessary for isolated axial neck pain without radiculopathy, myelopathy, or imaging-confirmed neural element compression

Relevant Codes:

ICD-10 (examples; verify plan-specific lists)

M54.2 Cervicalgia (neck pain) – often insufficient alone for surgery approval

M54.12 Radiculopathy, cervical region

M48.02- Spinal stenosis, cervical region (specify level as required by payer)

M50.1- Cervical disc disorder with radiculopathy (specify level/side per coding rules)

M47.12 Other spondylosis with myelopathy, cervical region

CPT/HCPCS (examples; verify coding based on operative note and payer rules)

63045 Laminectomy, facetectomy and foraminotomy; cervical, single vertebral segment (primary)

63048 Each additional segment (add-on to 63045–63047)

Note: Noncontiguous cervical segments may require separate primary coding and/or modifiers per CPT and payer policy. Verify final coding with your coding team.

Required Documentation:

- ☐ History and physical exam with detailed neurologic findings (motor strength, reflexes, sensory, gait/balance; signs of myelopathy if present)
- ☐ Duration, dates, and outcomes of conservative therapy (e6 weeks in last 6 months), or documentation of urgent indications for bypass
- ☐ Imaging reports (MRI preferred; CT/myelogram if MRI contraindicated) with level-specific correlation to symptoms and exam
- ☐ Pain severity and functional impairment measures (e.g., NDI, ADL/work limitations)
- ☐ Surgeon's rationale, planned levels (C2–C3 and C4–C5), and why noncontiguous levels are indicated
- ☐ Prior spine procedures and response (if any)
- ☐ Perioperative risk assessment/medical optimization as applicable

Limitations and Exclusions:

Isolated axial neck pain without objective neurologic deficit and without imaging-confirmed neural compression is generally not approved

Lack of or inadequate trial of conservative therapy (unless urgent red flags are documented)

Imaging that does not correlate with clinical findings or does not show compressive pathology at the requested levels

Requests made primarily for nonspecific pain, abnormal imaging alone, or prophylactic decompression without correlating symptoms

Summary Report

Determination:

Objective neurologic deficits and/or myelopathy, and

Imaging evidence of correlating compression at the proposed levels, and

Failure of a structured conservative treatment course (unless urgent red flags exist).

To strengthen the request, consider adding:

Detailed neuro exam demonstrating radiculopathy or myelopathy

MRI correlating compression at C2–C3 and C4–C5

Proof of 6 weeks conservative therapy and outcomes (or documentation of urgent indications)

Clear rationale for operating on two noncontiguous levels

Disclaimer: This guidance is based on generally accepted commercial prior authorization practices and publicly available information. It does not guarantee coverage or approval. Final determinations depend on the patient's specific benefit plan, medical necessity review, and the payer's most current policies. Always verify requirements with the specific commercial payer and the latest published criteria.